

Structured Clinical Endoscopy Report

Patient Name:	Sarah Jenkins	Patient ID:	PT-3841
Age / Gender:	45 / Female	Date of Procedure:	27091e6e59
Image Quality:	Blur Score: 693	Status:	Validated

Clinical Summary

Endoscopy demonstrates esophageal mucosal breaks of Mild severity (1.18% mucosal area coverage), indicating active reflux-induced Esophagitis.

Endoscopic Findings

Erythematous mucosal streaks and longitudinal erosions observed in the lower third of the esophagus above the gastroesophageal junction. Lesion coverage is 1.18%.

Condition Identified	Severity Classification	Inference Confidence
Reflux Esophagitis	Mild	85.0%

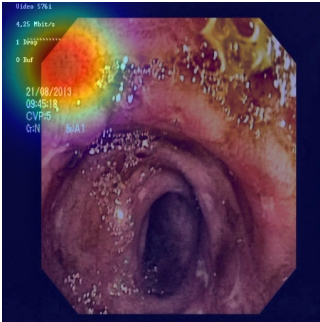
Comparative Multi-Stage Image Overlays



1. Original Frame



2. Enhanced (CLAHE)



3. Grad-CAM



4. Segmentation

Management Plan

- Initiate Esomeprazole 40mg PO daily (escalate to BID if symptoms persist/severe)
- Add Sucralfate 1g suspension QID for symptomatic relief of mucosal burning
- Lifestyle advice: elevate bed head by 6 inches, avoid lying down for 3 hours post meals, limit caffeine/chocolate/fats
- Follow up in 4-6 weeks to assess symptom resolution

Recommended Investigations

- Biopsy of gastroesophageal junction (if Barrett's or malignancy suspected)
- 24-hour pH impedance monitoring if refractory to PPI therapy

Cited Guidelines & Evidence

- Updated guidelines for the management of gastroesophageal reflux disease
- Los Angeles Classification of Esophagitis validity studies